

US Pharma & Biotech

US Pharma & Biotech: Insights from a physician on the ground at ASCO



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On Monday we hosted a KOL breakfast at ASCO 2026 with Dr. Fred Hirsch, a leading lung cancer clinician and researcher. The session covered the most important data presented over the weekend, with Dr. Hirsch offering his clinical perspective on a range of emerging treatments and trial readouts across early- and late-stage NSCLC.

Key topics included the LIBRETTO-432 adjuvant data for seliperatinib in RET-positive disease, the HARMONi-6 ivonescimab readout in squamous NSCLC, updated CROWN data for lorlatinib, next-generation ALK and KRAS inhibitors, TROP2-directed ADCs, and broader themes around molecular testing, treatment duration, and the evolving chronic disease framing of certain lung cancer subtypes.

- **HARMONi-6 was encouraging, but the critical open question is whether a China-only study translates to a global population.** The OS HR of 0.66 with manageable VEGF-related bleeding risk in squamous NSCLC was better than feared, but Dr. Hirsch cautioned that patient selection details were descriptive-only, age was capped at 75, and the Chinese population may not map cleanly to Western practice. He was clear that he would not anchor prescribing to a hazard ratio - approval plus a consistent HARMONi-3 readout in a global population are the two conditions needed for the drug to earn a place in the clinic; OS data from H3 are expected in H2 2026.
- **RET adjuvant data sets a new benchmark in early-stage targeted therapy.** LIBRETTO-432 delivered an HR of 0.17 for seliperatinib (LLY) in RET+ adjuvant NSCLC - among the most striking efficacy signals seen in this setting - though the key implementation challenge is detection, given ~70% of RET+ patients are never-smokers outside standard screening criteria.
- **Lorlatinib's bar remains extremely high, but Nuvalent has a credible path in lorlatinib-ineligible and lorlatinib-resistant patients.** Median PFS is still not reached at 7-year follow-up in CROWN - duration of therapy likely to be impressive; Nuvalent's 4th-generation ALK inhibitor shows ~26% response rates in lorla-relapsed patients and addresses a real unmet need in those with neurological toxicity or comorbidities.
- **KRAS is the next major battleground in first-line NSCLC.** Divarasib data in KRAS G12C first-line were described as potentially practice-changing, and the broader RAS space - including G12D and pan-RAS(ON) inhibitors from Revolution Medicines - is moving faster than many anticipated.
- **Sac-TMT is emerging as the most convincing TROP2 agent in NSCLC, but the all-comers vs. biomarker debate is unresolved.** Dr. Hirsch found the data compelling but remains philosophically conflicted around the absence of a companion biomarker strategy - a tension that may shape how the drug is ultimately positioned relative to Datroway.

BERNSTEIN TICKER TABLE

			1 Jun 2026		TTM	Adjusted EPS				Adjusted P/E (x)		
			Closing	Price	Rel.							
Ticker	Rating	Cur	Price	Target	Perf.	Cur	2025A	2026E	2027E	2025A	2026E	2027E
ABBV (AbbVie)	M	USD	212.93	225.00	(14.1)%	USD	9.98	14.29	15.93	21.3	14.9	13.4
AMGN (Amgen)	M	USD	329.13	335.00	(14.3)%	USD	21.85	22.49	22.82	15.1	14.6	14.4
BMJ (Bristol-Myers Squibb)	M	USD	54.95	58.00	(14.7)%	USD	6.15	6.31	6.02	8.9	8.7	9.1
LLY (Eli Lilly)	O	USD	1,082.20	1,300.00	18.1%	USD	24.20	37.41	47.99	44.7	28.9	22.5
GILD (Gilead)	O	USD	131.10	160.00	(9.5)%	USD	8.15	(0.69)	9.65	16.1	(191.1)	13.6
MRK (Merck)	M	USD	115.17	100.00	21.3%	USD	8.97	5.06	9.21	12.8	22.8	12.5
MRNA (Moderna)	M	USD	46.06	45.00	44.9%	USD	(7.27)	(8.39)	(4.18)	(6.3)	(5.5)	(11.0)
PFE (Pfizer)	M	USD	25.63	30.00	(19.4)%	USD	3.23	2.92	2.57	7.9	8.8	10.0
NUVL (Nuvalent)	O	USD	93.01	189.00	(3.9)%	USD	(5.85)	(5.87)	(5.84)	(15.9)	(15.8)	(15.9)
SMMT (Summit Therapeutics)	U	USD	15.71	7.70	(42.3)%	USD	(1.44)	(0.96)	(0.69)	(10.9)	(16.4)	(22.8)
BNTX (BioNTech)	M	USD	91.70	96.00	(32.8)%	EUR	(4.52)	(3.25)	(4.34)	(17.4)	(24.2)	(18.2)
RVMD (Revolution Medicines)	M	USD	163.68	151.00	286.9%	USD	(5.95)	(7.50)	(5.14)	(27.5)	(21.8)	(31.8)
INCY (Incyte)	M	USD	95.59	99.00	18.4%	USD	6.59	6.44	7.53	14.5	14.8	12.7
NBIX (Neurocrine)	O	USD	161.00	221.00	2.3%	USD	4.81	9.01	12.96	33.5	17.9	12.4
JAZZ (Jazz Pharma)	M	USD	232.64	229.00	86.7%	USD	(5.84)	13.76	9.69	(39.8)	16.9	24.0
SPX			7,599.96									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

NUVL, SMMT, BNTX, RVMD, INCY, NBIX, JAZZ estimate is Reported EPS; NUVL, SMMT, BNTX, RVMD, INCY, NBIX, JAZZ valuation is Reported P/E (x);

Source: Bloomberg, Bernstein estimates and analysis.

INVESTMENT IMPLICATIONS

US Biopharma: We rate LLY and GILD Outperform; ABBV, AMGN, BMJ, MRK, MRNA, PFE as Market-Perform.

US Biotech: We rate NUVL and NBIX Outperform; BNTX, RVMD, JAZZ and INCY as Market-Perform; SMMT as Underperform

DETAILS

RET+ Early-Stage Disease - LIBRETTO-432 (Selpercatinib)

The LIBRETTO-432 study in RET-positive patients moving into early-stage disease was highlighted as one of the most compelling data sets of the weekend, continuing a pattern set first by ADAURA in EGFR-mutant disease and ALINA in ALK-positive disease. Selpercatinib, a selective RET inhibitor with good CNS penetration, delivered a striking HR of 0.17 in a randomized adjuvant setting, with event-free survival curves already well-separated despite the EFS endpoint not yet being reached - viewed clinically as a clear positive. Notably, overall survival remains the endpoint patients care about most, and no OS data are yet available from this study, though EFS is a reasonable and accepted primary endpoint for this setting. Key open questions remain: the right duration of treatment (currently up to 3 years, with ctDNA monitoring a potential future tool, though early-stage patients shed less circulating DNA than advanced patients, making detection less straightforward); the absence of CNS protection data; and critically, how to detect these patients given that ~70% of RET+ patients are never-smokers who fall outside current lung cancer screening eligibility criteria, with an estimated ~30% diagnosed in Stage 1B-3A (though Dr. Hirsch noted this figure may skew high given his NYC practice). Molecular characterization also remains incomplete in practice - tissue requirements are still a barrier even in well-resourced settings, and liquid biopsy in early-stage disease is less reliable than in advanced disease. Several other studies in early-stage molecular subsets (including ROS and HER2-abnormal tumors) are underway.

Chinese RET Inhibitor (Lunbotinib)

A Chinese RET inhibitor also presented strong data with good brain penetrance, likely sufficient to support approval in China. However, per standard FDA practice, replication in a global Western population will be required before any US regulatory pathway can be pursued.

HARMONI-6 - Ivonescimab in Squamous NSCLC

Akeso's (covered by Rebecca Liang) China-only HARMONI-6 evaluated ivonescimab (a PD-1xVEGF bispecific) plus chemotherapy versus tislelizumab plus chemotherapy in squamous NSCLC, delivering an OS HR of 0.66 with results independent of PDL1 status and manageable toxicity. The bleeding risk historically associated with VEGF inhibition in squamous tumors (given their central, cavitating anatomy) was notably not a significant issue in H6, though questions were raised around potential patient selection effects - age was capped at 75, and eligibility data were described as descriptive-only rather than analytically defined. Dr. Hirsch emphasized that he would not anchor prescribing decisions to a specific hazard ratio threshold, and that an FDA-label from a positive global HARMONI-3 Ph3 would be prerequisites before he assessed real-world utility; patient-level factors including age, tumor location, comorbidities, hemorrhage/necrosis risk, and performance status all weigh heavily. He noted that slightly more than half of his squamous NSCLC patients are older than 65 - a substantial group whose risk-benefit calculus around VEGF inhibition requires individual assessment - though this proportion is gradually shifting younger. HARMONI-3, SMMT's global Ph3 trial, did not achieve statistically significant PFS during the recent interim analysis. Final PFS analysis and an OS interim analysis are projected to occur in the second half of this year.

PD-L1 Stratification and Emerging Treatment Assays

Dr. Hirsch noted that in PD-L1-high patients today he typically gives monotherapy IO rather than chemo combination, with this approach representing a minority of cases - reserved mainly for younger patients, those with higher tumor burden, or symptomatic patients where the benefit of adding chemotherapy justifies the toxicity. An emerging proteomic liquid biopsy assay (Prophet, by Oncohost) is in development and shows retrospective promise in identifying which patients require additional chemotherapy - but is not yet clinically established or approved.

Sacituzumab Tirumotecan (Sac-TMT) and TROP2 in NSCLC

Sac-TMT (an anti-TROP2 ADC) data presented at ASCO were described as more convincing than competing TROP2 agents, particularly in the Pembro plus or minus Sac-TMT setting in non-squamous NSCLC. Dr. Hirsch expressed some philosophical conflict around the all-comers approach given his background as a biomarker researcher, but acknowledged the data as compelling. ADC toxicity relative to chemotherapy remains difficult to generalize - patients react very differently - and clinical trials routinely exclude poor-performance-status patients, limiting real-world extrapolation.

CROWN Study Update - Lorlatinib and Next-Generation ALK Inhibitors (Nuvalent)

The CROWN study update for lorlatinib was described as setting a very high bar, with median PFS still not reached at 7-year follow-up - a striking contrast to the 6-7 month median survival seen in ALK-positive patients when Dr. Hirsch arrived in the US in 2000. Nuvalent's 4th-generation ALK inhibitor is notable for activity in lorlatinib-resistant tumors (~26% response rate in lorla-relapsed patients) and may carve a meaningful niche given that some patients are not candidates for lorlatinib due to neurological toxicity or comorbidities; Dr. Hirsch does not see weight gain or neurological issues frequently, partly because

ALK-positive patients represent only around 5-6 cases per year in his practice. He noted the cultural inertia around alectinib (a 2nd Gen ALK TKI) took years to overcome despite lorlatinib's (3rd Gen ALK TKI) superior data - implying that Nuvalent could face similar adoption challenges - though he believes continued data accumulation will ultimately drive uptake in lorlatinib-ineligible patients. Duration of therapy remains an open question with no clear answer; ctDNA and liquid biopsy monitoring are hoped-for future tools, but for now patients are generally reluctant to stop therapy if tolerability is acceptable - a point Dr. Hirsch framed with reference to his own experience as a cancer survivor treated two decades ago, where he received a wide range of recommendations but ultimately chose to continue treatment.

ROS1, Exon 20, and KRAS G12C

ROS1 is too rare in Dr. Hirsch's practice for meaningful comment. Exon 20 insertions remain predominantly an Asian patient population story, with sunvozertinib-based combinations worth monitoring for the future (presented by John Heymach). KRAS G12C data in first-line (divarasib) were described as potentially practice-changing, given the higher prevalence of KRAS vs. ROS1 or Exon 20. The broader KRAS/RAS space - including G12D and pan-RAS(ON) inhibitors from Revolution Medicines - is moving very rapidly with encouraging early signals, though full data are needed.

PD-1xVEGF Combined with Novel Mechanisms (Integrin Beta-6 / Pfizer)

A potential combination of PD-1xVEGF with integrin beta-6 (potentially Pfizer-driven) was flagged as an interesting mechanistic combination but was noted to be lacking a biomarker-driven development strategy - a gap Dr. Hirsch views as a significant omission. More data are required before clinical utility can be assessed.

NCCN Guidelines and Clinical Practice Philosophy

Dr. Hirsch expressed confidence that NCCN guidelines will continue to be updated on a rolling basis given the pace of data generation, typically on a months rather than days or years cadence - a function of the NCCN model, which convenes an expert group quickly and grades evidence more efficiently than many other guideline frameworks. On the broader question of clinical decision-making, he emphasized that treatment decisions are never driven by hazard ratios alone - patient-level factors, tolerability, expectations, and comorbidities all inform a shared decision-making process, and doctors should be treating patients rather than data points. He grounded this philosophy partly in his own experience as a cancer survivor treated two decades ago, when his survival curve did not look encouraging and he received a wide range of recommendations; his wife's observation that "median survival is for statisticians, not for individual patients" captures how he approaches conversations with his own patients today. Looking further ahead, he noted that for certain molecular subtypes, lung cancer is approaching a chronic disease paradigm - patients may now live 7-10 years with their diagnosis - making survivorship programs an increasingly urgent area of focus, one the lung cancer community has not yet fully prioritized; these programs need to address physical, psychological, family, and financial impacts in equal measure.

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